HEALTH BEACON

CMSRN EXAM PREP

Practice Exam 01 - Holistic Patient Care

19 scored practice questions | Printable Practice Exam

Instructions	Select the single best answer for each question. An answer key with explanations begins after the question section.

PART A - QUESTIONS

1.

Scenario

A patient newly diagnosed with colorectal cancer is alert, has decision-making capacity, and is meeting with the care team. The patient's adult daughter answers most questions. The patient says quietly, "I want to make my own decisions, but I still want her here."

Which response by the nurse best supports patient-centered care?

A) Ask the daughter to leave so the patient can speak without family influence.

B) Continue the discussion with the daughter because the patient wants her present.

C) Acknowledge the patient's request and ask how the patient wants the daughter involved in future discussions.

D) Postpone further discussion until the patient and daughter agree on who will make decisions.

2.

Scenario

A patient with a new ileostomy is preparing for discharge. The patient says, "There are too many steps. I'm afraid I'll do something wrong at home." Pain is controlled, and the patient is willing to practice.

Which teaching approach is most appropriate?

A) Divide the skill into small steps, demonstrate each part, and have the patient perform a return demonstration.

B) Give the patient a detailed instruction packet to review independently after discharge.

C) Ask a family member to learn the procedure and perform all pouch changes initially.

D) Repeat the complete explanation until the patient can recite the sequence from memory.

3.

Scenario

A patient hospitalized with advanced heart failure has repeated admissions for dyspnea. The patient says, “I still want treatments that may help me live longer, but I need better relief and help talking with my family about what matters to me.”

Which nursing action best addresses the patient’s request?

A) Explain that comfort-focused services begin only after disease-directed treatment stops.

B) Recommend hospice enrollment because repeated admissions indicate a prognosis of less than 6 months.

C) Ask the family to decide whether the patient should continue disease-directed treatment.

D) Request a palliative care consultation for symptom management and goals-of-care support alongside current treatment.

4.

Scenario

A patient with limited English proficiency is scheduled for a colonoscopy. The clinician arrives to discuss the procedure and obtain informed consent. The patient’s teenage child offers to interpret and says, “I know what my parent wants.”

What should the nurse do?

A) Use the child to interpret because the child already understands the patient’s preferences.

B) Arrange a qualified medical interpreter and speak directly to the patient during the discussion.

C) Ask the clinician to use short English sentences while the child clarifies unfamiliar words.

D) Provide the English consent form and ask the patient to sign after the child reviews it.

5.

Scenario

A patient with hearing loss returns from imaging without hearing aids in place. During bedside shift report, the patient says, “Please face me when you talk, and write down changes to my plan.”

Which action best honors the patient’s communication preferences?

A) Ask the family to repeat the report later when the patient is less tired.

B) Speak more loudly from the doorway and provide the standard discharge packet.

C) Complete report outside the room to avoid embarrassing the patient about hearing loss.

D) Help replace the hearing aids, face the patient, provide a written summary, and confirm the preferred approach.

6.

Scenario

A patient will self-administer enoxaparin after discharge for treatment of a venous thromboembolism. After watching the nurse demonstrate, the patient nods and says, “That seems easy enough.”

How should the nurse best evaluate whether the teaching was effective?

A) Ask the patient to perform an injection using the practice supplies and explain safe sharps disposal.

B) Ask whether the patient feels confident about giving the injection without assistance.

C) Have the patient read the medication handout aloud before leaving the hospital.

D) Request that a family member sign the instructions as proof that teaching occurred.

7.

Scenario

A patient with progressive neurologic disease lacks decision-making capacity. The record contains a valid advance directive stating that the patient does not want intubation if the disease progresses. During a care conference, the spouse asks the team to “do everything, including the breathing tube.”

What is the nurse’s best response?

A) Tell the spouse that family requests automatically replace written directives once a patient loses capacity.

B) Avoid discussing the conflict because only the provider may address advance directives.

C) Acknowledge the spouse’s distress, identify the documented preference, and notify the provider so the team can clarify a goal-concordant plan.

D) Ask the spouse to sign a new advance directive on the patient’s behalf before the conference continues.

8.

Scenario

A patient with heart failure says, “I know I should buy low-sodium foods, but the food pantry gives me whatever is available, and I cannot afford the products on this list.”

Which nursing intervention best supports a realistic health-promotion plan?

A) Emphasize that preventing readmission requires the patient to follow the prescribed list exactly.

B) Teach the patient to avoid all canned and packaged foods regardless of availability.

C) Delay dietary counseling until the patient can obtain food from a different source.

D) Involve the dietitian and care coordinator to identify affordable choices, pantry resources, and benefits for which the patient may qualify.

9.

Scenario

A patient on a medical-surgical unit says that repeated nighttime interruptions have prevented sleep for 3 nights. The patient is angry and says, “No one listens here. I want to leave.”

What is the nurse’s best initial response to the concern?

A) Explain that interruptions are unavoidable because hospitalized patients require frequent monitoring.

B) Invite the patient to describe the most disruptive events, acknowledge the impact, and collaborate on changes that can be made safely.

C) Give the patient the hospital grievance form and ask the patient to complete it before further discussion.

D) Document that the patient is dissatisfied and wait for the patient-experience department to respond.

10.

Scenario

A patient receiving hospice care for pancreatic cancer is no longer able to describe pain. The patient is grimacing, guarding the abdomen, and moaning during repositioning. The daughter says, “Please do not give morphine. I’m afraid of addiction.”

Which action by the nurse is most appropriate?

A) Withhold the opioid until the patient can provide a numeric pain rating.

B) Assess behavioral indicators, administer the prescribed opioid according to the comfort plan, and reassess while addressing the daughter’s concern.

C) Substitute a placebo so the family can observe whether the behaviors improve.

D) Wait for the hospice prescriber to speak with the daughter before treating the symptoms.

11.

Scenario

During handoff, a staff member describes a patient with obesity as “noncompliant” before discussing the patient’s transportation limitations, medication cost, or health goals.

Which action by the receiving nurse best reduces the effect of implicit bias on care?

A) Reframe the discussion with objective findings and explore the patient’s barriers and goals without judgment.

B) Retain the label in the care plan so all staff know adherence may be difficult.

C) Avoid asking about weight-related goals because the topic may make the patient uncomfortable.

D) Tell the patient that adherence must improve before the team can offer additional resources.

12.

Scenario

An older adult admitted after a minor fall reports two previous falls at home. The patient takes several medications, has dim hallway lighting, and has not had a vision examination in 4 years. The patient wants to remain independent.

Which plan best promotes the patient’s stated goal?

A) Recommend that the patient stop walking without family supervision.

B) Focus on purchasing a medical alert device before addressing other risks.

C) Collaborate on a home-safety plan, medication review, vision follow-up, and a fall-reduction goal chosen by the patient.

D) Advise the patient to move to an assisted living facility after discharge.

13.

Scenario

A patient with heart failure is taught to weigh at the same time every morning and call the clinic the same day for a gain of 2 lb (0.9 kg) in 24 hours or 5 lb (2.3 kg) in a week. The patient’s weight was 178 lb yesterday and is 181 lb today.

Which patient statement shows correct understanding of the plan?

A) “I will skip my next meal and check my weight again tomorrow.”

B) “I will take an extra diuretic now even though it was not prescribed.”

C) “I will call the clinic today even if I do not feel short of breath.”

D) “I will wait until my weekly gain reaches 5 pounds before reporting it.”

14.

Scenario

A patient with prediabetes and knee osteoarthritis wants to become more active but cannot afford a gym membership. The patient enjoys being outdoors and says that long walks increase knee pain.

Which nursing response best supports sustainable health promotion?

A) Help the patient choose a short, low-impact activity goal and identify free local or home-based options that fit the patient's interests.

B) Explain that meaningful diabetes prevention requires at least 60 minutes of uninterrupted exercise daily.

C) Recommend postponing exercise until the knee is completely pain-free.

D) Provide a list of commercial fitness centers so the patient can compare membership prices.

15.

Scenario

After a hospitalized patient dies, the family requests that the body not be washed until a religious leader arrives. No medical examiner review or organ donation procedure is pending, and the requested delay can be accommodated within hospital policy.

What should the nurse do?

A) Explain that standard post-mortem care must begin immediately for every patient.

B) Honor the request, coordinate the timing with appropriate staff, and document the family's preference.

C) Ask the family to perform all post-mortem care without staff assistance.

D) Remove all tubes and lines before discussing the request with the religious leader.

16.

Scenario

A transgender patient states a name and pronouns that differ from those on the insurance record. The nurse hears a staff member joke about the difference at the nurses' station.

Which action best promotes inclusive care?

A) Use only the legal name until the insurance record is formally changed.

B) Ignore the joke unless the patient reports hearing it.

C) Ask the patient to explain the gender identity to each staff member individually.

D) Use the stated name and pronouns, address the disrespectful behavior, and follow policy to make the preference visible to the care team.

17.

Scenario

A competent patient tells the nurse, "Do not update my brother about my condition. I will speak with him when I am ready." Later, the brother calls and demands the latest test results, saying he is the patient's primary support person.

How should the nurse respond?

A) Share only results that are not considered serious.

B) Provide the results because the brother identifies himself as the primary support person.

C) Protect the patient's confidentiality and ask the patient what, if anything, may be shared.

D) Tell the brother that no information can ever be shared with family members.

18.

Scenario

An older adult with limited health literacy is discharged with new morning furosemide and potassium prescriptions. The patient takes several other medications and appears confused by the printed list.

Which nursing intervention is most appropriate?

A) Create a plain-language schedule tied to the patient's routine, review one section at a time, and use teach-back.

B) Read the entire medication list aloud slowly and ask whether the patient understands.

C) Ask the patient to memorize the medication names before discharge.

D) Tell the patient to rely on the pharmacy labels and call later if questions arise.

19.

Scenario

A patient with a catastrophic neurologic injury meets the hospital's clinical trigger for referral to the organ procurement organization. The family has not raised the topic of donation, and the patient's donor eligibility has not been determined.

What is the nurse's appropriate action?

A) Ask the family for donation consent before making the referral.

B) Notify the organ procurement organization according to hospital policy and continue patient care without promising eligibility.

C) Tell the family that the patient qualifies as an organ donor.

D) Wait until the family requests information about donation.

PART B - ANSWER KEY AND EXPLANATIONS

Question numbering below corresponds to the scored questions in Part A.

1	C	2	A	3	D	4	B	5	D
6	A	7	C	8	D	9	B	10	B
11	A	12	C	13	C	14	A	15	B
16	D	17	C	18	A	19	B		

1. Correct: C - Acknowledge the patient's request and ask how the patient wants the daughter involved in future discussions.

A capable patient remains the decision-maker and may define the family's role. Clarifying that preference supports autonomy without unnecessarily excluding a desired support person.

2. Correct: A - Divide the skill into small steps, demonstrate each part, and have the patient perform a return demonstration.

Chunking information and using return demonstration reduce cognitive burden and verify that the patient can perform the skill rather than merely recognize or recite it.

3. Correct: D - Request a palliative care consultation for symptom management and goals-of-care support alongside current treatment.

Palliative care can be provided concurrently with disease-directed treatment and can address symptoms, communication, and patient-defined goals. Hospice eligibility cannot be inferred from admission frequency alone.

4. Correct: B - Arrange a qualified medical interpreter and speak directly to the patient during the discussion.

A qualified interpreter supports accurate, confidential communication and informed decision-making. A minor family member should not replace a qualified interpreter for a consent discussion.

5. Correct: D - Help replace the hearing aids, face the patient, provide a written summary, and confirm the preferred approach.

Restoring the patient's communication aid and using the requested visual and written methods directly honors the patient's stated preference and supports meaningful participation.

6. Correct: A - Ask the patient to perform an injection using the practice supplies and explain safe sharps disposal.

Return demonstration plus explanation evaluates both psychomotor skill and understanding. Confidence, reading ability, or a signature does not establish safe performance.

7. Correct: C - Acknowledge the spouse's distress, identify the documented preference, and notify the provider so the team can clarify a goal-concordant plan.

The nurse should advocate for the patient's documented wishes while responding respectfully to the surrogate's distress and promptly engaging the team to resolve the conflict. A surrogate does not create a new advance directive for an incapacitated patient.

8. Correct: D - Involve the dietitian and care coordinator to identify affordable choices, pantry resources, and benefits for which the patient may qualify.

A feasible plan addresses the social and financial barriers affecting food choices. Repeating an unaffordable prescription does not support sustainable self-management.

9. Correct: B - Invite the patient to describe the most disruptive events, acknowledge the impact, and collaborate on changes that can be made safely.

Active listening, validation, and shared problem-solving are appropriate first steps in service recovery. A grievance process may remain available but should not replace an immediate response to the concern.

10. Correct: B - Assess behavioral indicators, administer the prescribed opioid according to the comfort plan, and reassess while addressing the daughter's concern.

Behavioral signs are appropriate evidence of pain when self-report is unavailable. Treating and reassessing distress are consistent with hospice goals; fear of addiction should be addressed without delaying indicated comfort medication.

11. Correct: A - Reframe the discussion with objective findings and explore the patient's barriers and goals without judgment.

Objective language and direct exploration of barriers reduce the chance that an unexamined stereotype will shape care. A stigmatizing label can obscure transportation, cost, and patient-priority factors.

12. Correct: C - Collaborate on a home-safety plan, medication review, vision follow-up, and a fall-reduction goal chosen by the patient.

Multifactorial fall prevention addresses modifiable risks while preserving the patient's goal of independence. Restricting mobility or recommending relocation without shared assessment may cause harm and disregard preferences.

13. Correct: C - "I will call the clinic today even if I do not feel short of breath."

The 3-lb overnight increase exceeds the explicitly taught same-day reporting threshold. Symptoms need not be present, and the patient should not independently change medication doses.

14. Correct: A - Help the patient choose a short, low-impact activity goal and identify free local or home-based options that fit the patient's interests.

A patient-selected, affordable, tolerable goal is more likely to be sustained. An all-or-nothing requirement or postponing activity does not build feasible health behavior.

15. Correct: B - Honor the request, coordinate the timing with appropriate staff, and document the family's preference.

Cultural and spiritual preferences should be incorporated into post-mortem care when they do not conflict with required procedures or policy. Immediate standardized care is not necessary under the conditions given.

16. Correct: D - Use the stated name and pronouns, address the disrespectful behavior, and follow policy to make the preference visible to the care team.

Respectful language, correction of biased behavior, and reliable communication of the patient's preference promote dignity and consistent inclusive care while preserving required legal documentation.

17. Correct: C - Protect the patient's confidentiality and ask the patient what, if anything, may be shared.

A capable patient controls disclosure of health information. Clarifying any authorized scope of sharing respects both confidentiality and the patient's choice about family involvement.

18. Correct: A - Create a plain-language schedule tied to the patient's routine, review one section at a time, and use teach-back.

Plain language, manageable information segments, and teach-back reduce medication confusion and verify understanding. A yes-or-no question or passive reliance on labels does not assess comprehension.

19. Correct: B - Notify the organ procurement organization according to hospital policy and continue patient care without promising eligibility.

Timely referral allows the designated organization to evaluate potential donation and manage the donation process. The bedside nurse should not determine eligibility or independently initiate a consent request.